

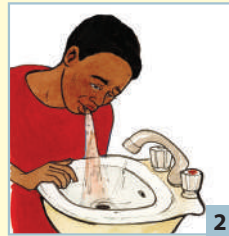
HOW TO COLLECT A GOOD SPUTUM SPECIMEN FOR TB TESTING

Aim to collect sputum in the early morning. This improves the chance of an accurate result. However, avoid missing the opportunity to collect sputum anytime during a consultation.

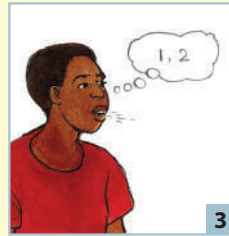
- Explain that a good quality sputum specimen is important to make an accurate diagnosis of TB.
- Advise to avoid putting saliva or nasal secretions into specimen jar. Sputum is the secretion that comes from deep within the lungs and a forceful cough is needed to bring it up for collection.
- If observing sputum collection, health worker to use mask (N95/FFP2) in well ventilated area. Stand behind patient and check air stream (fan, air conditioner) is coming from behind back to avoid exposure.
- Explain how to collect a good sputum specimen:



- Ensure collection area is well ventilated and private.
- Use a designated sputum collection area if available.



- Rinse mouth with water to remove food, mouth wash or medication.



- Breathe in and out deeply two times.
- Have an open specimen jar ready.
- Keep the jar sterile (clean), avoid touching inside it.



- On the third breath, give a strong cough.
- Cough 5-10mL (1-2 teaspoons) sputum into the jar.
- You may need several coughs to get at least 5mL.
- Avoid putting saliva/nasal secretions into jar.



- Replace lid and screw on tightly to prevent leaking.
- Give to health worker.



- Wash your hands after sputum collection.

If patient unable to produce sputum, use nebuliser to induce sputum

- Health worker to wear a N95/FFP2 respirator mask in a well ventilated area. Explain process to patient: follow same steps as above only use nebuliser to help produce sputum.
- Add 5mL **3% saline** to nebuliser and nebulise at 8L/min for 10 minutes or until sputum coughed up. While nebuliser running, ask patient breathe in and out deeply 2-3 times, followed by a strong cough to try and bring up sputum from deep within the chest. If able to generate sputum, ask patient to remove mask and cough it into the specimen jar, then continue with nebulisation. Repeat until at least 5mL sputum collected.
- If no sputum coughed up by 10 minutes, repeat nebulisation once.

Prepare specimens for transport to the laboratory:

- Check specimens are adequate: if patient unable to produce 5mL (1 teaspoon) but quality of sputum is still good, still send specimens to laboratory. If quality of specimen is inadequate, see below.
- Ensure lid is closed tightly and correctly, and that the specimen jar is correctly labelled as above. Wash hands after handling specimens.
- If room temperature is > 25°C or transport delayed for > 24 hours, store in refrigerator (2-8°C). Keep cool but do not freeze.
- Complete request form and advise patient to return for results in 2 days. If patient HIV-positive, indicate this on request form.

If specimen inadequate:

- If specimen is inadequate and of poor quality after repeated attempts, discard used jar in medical waste bin and give patient new labelled specimen jar. Instruct on how to collect sputum at home:
 - Collect sputum specimen early in the morning after waking up, before eating or taking any medications. Collect sputum specimen outside home. Follow the same steps tried above.
 - Once collected, protect sputum specimen sample from heat and light. Keep at room temperature and bring to the clinic as soon as possible.
- If specimen from home is adequate, prepare for transport to laboratory (above). If still not adequate, refer to hospital for further investigation or to a doctor for chest x-ray and review.